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Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

RE: CMS-0042-NC, CMS Request for Information on Health Technology Ecosystem

Homeward Health, the only AI-native healthcare platform built specifically for rural populations, appreciates the opportunity to respond to the Centers for Medicare & Medicaid Services' (CMS) Request for Information on the Health Technology Ecosystem. Homeward is a technology-enabled provider organization that delivers population-level results at scale for 100,000 rural seniors by providing patients with targeted interventions and providers with AI-powered decision support. Homeward sees firsthand the enormous potential—and persistent barriers—facing the deployment of digital health solutions in communities that need them most. We offer comments grounded in operational experience, technology-enabled care models, and our deep commitment to expanding rural health access.

In the sections that follow, (I) we first provide a brief description of Homeward Health, followed by (II) a summary of our comments and (III) a detailed response to the question most salient to our work and expertise in rural healthcare transformation: PR-1. These recommendations reflect our desire to work closely with CMS to build a digital health ecosystem that supports meaningful care transformation, particularly in the communities where access gaps have historically been the most severe — like in rural America.

I. About Homeward Health

Homeward Health is a healthcare technology company committed to expanding access for and delivering high-quality, technology-enabled care to individuals living in rural communities across America. Through numerous partnerships with health plans and providers, we deploy a hybrid care model that combines in-person services with virtual care, remote monitoring, and integrated care navigation and coordination. Our infrastructure is designed to meet the unique needs of rural populations by overcoming geographic, technological, and systemic barriers to care. Our mission is closely aligned with CMS's priorities of expanding digital health access, advancing value-based care (VBC), and better meeting patient needs all while driving efficiency and reducing the cost of care.

II. Summary of Comments

In this response, we focus on providing a comprehensive and actionable set of policy recommendations rooted in Homeward's deep operational experience in reaction to question PR-1, which asks how CMS can better support rural providers in adopting digital health tools. Our comments are informed by field experience through our direct work with rural partners and our technology-supported care model alongside a foundational commitment to improving rural health outcomes through innovation.

Our response outlines a comprehensive framework for CMS action, structured around six core challenges that rural providers face in adopting digital health products. These are:

1. Lack of incentives and capital in Fee-For-Service dominated environments
2. Outdated technical infrastructure and limited IT support
3. Low digital literacy and limited technical expertise among rural providers
4. Fragmented health IT systems and interoperability challenges
5. Low awareness of available tools due to limited vendor engagement
6. Distrust of tools that add workflow burden or reduce autonomy

For each of these six challenges, we provide a detailed problem analysis. For the first three, we also offer a set of actionable CMS policy recommendations. For the latter three, we intend to engage further with CMS and stakeholders to co-develop effective solutions.

III. PR-1: How Can CMS Encourage Rural Providers to Adopt Digital Health Products?

Rural providers face a constellation of systemic barriers that hinder the adoption of digital health tools, including those powered by generative AI (GenAI). These challenges — from misaligned payment models and inadequate capital to workforce shortages and outdated infrastructure — are not merely operational hurdles; they are structural failures that threaten quality and access across the healthcare system. Even clinically effective technologies remain out of reach when fee-for-service reimbursement discourages investment, IT capacity is limited, and tools are poorly integrated into existing workflows. Clinician concerns around workflow burden, autonomy, data privacy, and patient education further suppress adoption. Collectively, these barriers disproportionately restrict rural providers' ability to modernize care delivery, expand access, and manage population health effectively.

Rural healthcare will not benefit from digital transformation through passive diffusion. It will require proactive, sustained intervention. CMS is uniquely positioned to correct these market failures — by realigning incentives, reducing adoption risk, and providing the infrastructure, technical support, and regulatory clarity needed to bring rural care into the digital and AI-driven future. The following recommendations outline a path forward.

1. Lack of Incentives and Capital in Fee-for-Service Dominated Environments

Most rural providers operate under traditional Medicare and Medicaid fee-for-service models that reward volume over value, prioritizing in-person visits and procedures over preventive, team-based, or virtual-first care. This misalignment leaves providers with little financial incentive to invest in digital tools, even when those tools can reduce hospitalizations, improve engagement, or enable more efficient care delivery.

The stakes are especially high in rural settings, where chronic disease, behavioral health needs, and barriers to in-person care are more acute compared to urban areas. Digital and team-based models — such as remote monitoring, virtual chronic care management, and EHR-integrated care planning — offer a powerful means to close these gaps. Emerging AI-enabled technologies, like ambient documentation tools, virtual triage, and predictive risk stratification, also hold promise for extending rural care capacity but only if providers can access the infrastructure, training, and support needed to deploy them safely and effectively.

Yet small, independent rural clinics often cannot afford the upfront costs of adoption, including infrastructure, software, training, and workflow redesign. Without targeted reimbursement reforms or upfront investment, rural providers are unable to modernize care delivery — even as digital-first approaches are uniquely suited to address the health needs of their communities. CMS has a critical role to play in correcting these structural barriers by realigning payment, reducing adoption risk, and enabling rural practices to build the digital foundation necessary for sustainable, effective care. The recent Rural Patient Monitoring Access Act (S.1535 / H.R.3108) introduced in the U.S. Senate is an example of the kind of reform needed – the measure raises reimbursement of rural remote patient monitoring technologies to parity with urban settings. It offers a compelling model for how targeted policy can unlock digital innovation in the communities that need it most.

Recommended CMS Actions:

1. <u>Develop new rural-specific reimbursement policies</u>
    a. **Offer add-on payments within the Medicare Physician Fee Schedule (MPFS) for key digital health services**, including remote physiologic monitoring, asynchronous communication, and tech-enabled patient navigation. This should also include reimbursement for additional underutilized CPT codes critical to digital care, such as (1) Chronic Care Management (CCM), (2) Principal Care Management (PCM), (3) Remote Therapeutic Monitoring (RTM), and (4) Interprofessional Consultations (e-Consults).
    b. **Establish bundled payments** for high-frequency digital interventions in chronic care management, making them administratively simpler for rural clinics to implement.

   c. **Introduce a Digital First Access Payment Program**, similar in structure to Graduate Medical Education (GME) payments, that allocates annual lump-sum payments to rural practices investing in first-time digital infrastructure — something which becomes much more accessible through the advent of GenAI.
2. More robustly incentivize adoption of telehealth in rural communities
   a. **Permanently eliminate originating site and geographic restrictions that limit access to telehealth in rural settings**. This includes formally codifying the home as a permanent, eligible site of care under Medicare, which currently allows it only through temporary waivers extended by Congressional action. Without statutory codification in the Social Security Act, these flexibilities remain vulnerable to expiration, limiting provider confidence and long-term investment in home-based virtual care infrastructure.
   b. **Establish permanent reimbursement parity for audio-only visits** given persistent broadband limitations in many rural communities, especially for behavioral health, chronic care check-ins, and other clinically appropriate services. Doing so would better reflect the technological realities of rural environments and further incentivize provider investment in virtual care infrastructure.
   c. **Fully include Federally Qualified Health Centers (FQHCs) and Rural Health Clinics (RHCs) in telehealth parity and modernization policies** to avoid excluding core safety-net providers that serve a large share of rural populations.
   d. **Explore the creation of a unified, cross-program telehealth policy with payment parity across Medicare Parts A and B, as well as Medicaid**, to reduce administrative complexity and encourage broader state Medicaid adoption — potentially supported by enhanced federal matching (FMAP) incentives.

2. Low Digital Literacy and Limited Technical Expertise Among Rural Providers

Rural providers often operate in environments without dedicated information technology (IT) personnel or digital transformation leadership. Unlike integrated delivery networks or large urban hospitals that can staff CIOs and implementation teams, rural clinics may rely on generalists or external consultants for all technology decisions. This lack of capacity makes it difficult to confidently assess vendors, navigate implementation logistics, or redesign clinical workflows. In many cases, technology adoption efforts are abandoned or underutilized due to these deficits, which compound other systemic challenges such as staff shortages and low financial margins. This barrier is well-documented in federal studies: the Office of the National Coordinator for Health IT (ONC) has highlighted that, due to internal resource constraints, small and rural practices face disproportionate barriers to electronic health record (EHR) optimization

and adoption of digital tools like those supporting clinic decision making and diagnostics, clinical documentation and ambient scribing, and AI in patient engagement and virtual care.

Recommended CMS Actions:

1. Modernize and reestablish a technical assistance infrastructure focused on rural digital health transformation
    a. **Fund the creation of 'Rural Health IT Resource Centers'** modeled after the HITECH Act's Regional Extension Centers (RECs). These centers would offer vendor-neutral procurement support, workflow design, digital technology (AI-inclusive) integration assistance, and hands-on digital coaching to rural providers.
        i. *RECs played a key role in supporting over 100,000 priority, underserved, primary care providers through the adoption and meaningful use of EHRs. Evaluations of REC programs found that they significantly increased EHR adoption rates and improved clinical documentation quality.*
    b. **Launch a 'Digital Health Navigators' fellowship**, which would deploy a national cohort of health IT fellows trained in implementation science, clinical informatics, and rural care delivery. These navigators would embed temporarily at rural sites to provide operational support, develop digital roadmaps, and serve as trusted intermediaries between providers and technology vendors.
    c. **Launch a rural AI enablement program** that provides small and rural providers with practical tools to assess, procure, and implement AI technologies responsibly. These resources would reduce procurement risk, simplify implementation, and ensure new AI tools align with rural realities like low bandwidth, generalist staffing, and fragmented infrastructure. *This could include:*
        i. Standardized AI readiness and risk assessment toolkit
        ii. CMS-endorsed "Trusted AI Vendor" designation, certifying tools that are explainable, interoperable, and tested for bias and safety in rural contexts

3. Distrust of Tools That Add Workflow Burden or Reduce Autonomy

Years of exposure to poorly designed digital tools (often deployed for billing or compliance) have left many rural clinicians skeptical of new technology. Tools that slow them down, require duplicative documentation, or dictate clinical workflows are often resisted. This distrust is especially acute in environments where provider burnout (caused by physician and skilled labor shortages) is already high. Unlike larger systems that can absorb workflow disruption through administrative or IT support, rural practices often feel the effects of even small inefficiencies more acutely. This has contributed to a pervasive sense among rural providers that digital tools are burdens rather than supports. To promote adoption, rural providers must see technology as an

asset to clinical care—not a source of friction or control. Building this trust requires a significant cultural and operational shift that centers the voices of clinicians and aligns design incentives with practical realities.

Recommended CMS Actions:

Prioritize user-centered design and provider co-creation in certification and funding processes: tools funded or approved through CMS programs should demonstrate clinical utility *and* provider usability.

1. **Establish digital sandbox models** through CMMI that allow rural providers to test tools in controlled environments and provide structured usability feedback prior to full implementation. These sandboxes would serve as proving grounds to surface usability issues early, helping vendors adapt before wide-scale rollout.
2. **Expand agency support for real-world usability research focused on rural workflows.** Funding should prioritize mixed-methods studies that examine how tools impact provider time, clinical decision-making, and overall satisfaction in low-resource environments.
3. **Include provider experience and autonomy metrics—such as documentation time reduction, EHR task load, and alignment with scope of practice—within its vendor evaluation frameworks.** Vendors should be rewarded for tools that streamline workflows and penalized for those that increase administrative complexity. CMS could also consider pilot testing a voluntary "Rural Usability Seal" for tools that meet threshold performance standards in rural settings, akin to the FDA's software precertification framework.

Sections 4–6 provide additional detail on key barriers to digital health adoption among rural providers. While these sections do not yet include detailed CMS policy recommendations, they highlight priority areas where we welcome further dialogue and collaboration.

4. Outdated Technical Infrastructure and Limited IT Support

Even when rural providers are motivated to adopt digital tools, many lack the infrastructure to do so. Broadband limitations, outdated hardware, on-premise EHRs, and cybersecurity vulnerabilities create high barriers to entry. Additionally, these providers often operate without dedicated IT support, making it difficult to manage even routine upgrades or troubleshoot implementation issues. Without core infrastructure and support systems, even well-intentioned efforts to adopt digital tools frequently fail or remain underutilized.

5. Fragmented Health IT Systems and Interoperability Challenges

While Section 3 above addressed the absence of foundational digital infrastructure — such as broadband access, modern hardware, and IT staffing — this section focuses specifically on the fragmentation and incompatibility of the digital systems that do exist. Even in technically connected settings, rural providers often use disjointed platforms lacking standardization, which makes it difficult to integrate new tools or coordinate care across settings.

More specifically, rural providers frequently rely on disparate, unintegrated EHR systems with limited interoperability. Many lack participation in regional Health Information Exchanges (HIEs) or access to FHIR-compatible platforms. These gaps impede data sharing, prevent closed-loop referrals, and make it difficult to layer third-party tools onto existing systems. This fragmentation not only undermines care coordination but also creates implementation and compliance burdens that further discourage digital adoption.

6. Low Awareness of Available Tools Due to Limited Vendor Engagement

Digital health vendors tend to prioritize urban and academic medical centers when deploying their solutions, in part because of better reimbursement, faster procurement timelines, and larger contract sizes. This underinvestment in rural outreach results in poor awareness among rural providers of available tools that could meet their specific needs. In many cases, providers are unaware of innovations in areas such as asynchronous triage, low-bandwidth video platforms, or AI-powered documentation tools. The result is a deep digital awareness divide. Without structured channels for rural provider education and market exposure, high-potential tools are often not reaching the communities that could benefit most from them.

Conclusion

The transformation of rural healthcare through digital innovation is not only possible but imperative. By addressing structural barriers through coordinated policy action, CMS can build a future where digital health is accessible and effective across every zip code.